Degarelix Therapy

INDICATIONS FOR USE:

INDICATION	ICD10	Regimen Code	HSE approved reimbursement status*
Treatment of adult male patients with advanced hormone-dependent prostate cancer	C61	00481a	N/A

* This applies to post 2012 indications

TREATMENT:

The starting dose of the drugs detailed below may be adjusted downward by the prescribing clinician, using their independent medical judgement, to consider each patients individual clinical circumstances.

Degarelix 240mg is administered as two consecutive subcutaneous (SC) injections of 120mg each on day 1. The first maintenance dose of degarelix 80mg is administered 28 days after the starting dose and continued every 28 days until disease progression or unacceptable toxicity develops.

Day	Drug	Dose	Route	Cycle
1	Degarelix	[a]240mg	SC	1 only
1	Degarelix	80mg	SC	2 onwards
[a] Degarelix 240mg is administered as two consecutive SC injections of 120mg each				
Degarelix is administered as a SC injection in the abdominal region. The injection site should vary periodically. Injections should be given in areas where the patient will not be exposed to pressure e.g. not close to waistband or belt and not close to the ribs				

ELIGIBILITY:

- Indication as above

CAUTIONS:

- Androgen deprivation therapy may prolong the QT interval. In patients with a history of or risk factors for QT prolongation over 450 msec, and in patients receiving concomitant medicinal products that might prolong the QT interval, physicians should assess the benefit risk ratio including the potential for Torsade de pointes prior to initiating treatment.

EXCLUSIONS:

- Hypersensitivity to degarelix or any of the excipients

PRESCRIPTIVE AUTHORITY:

The treatment plan must be initiated by a Consultant with expertise in the treatment of prostate carcinoma.

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TESTS:

Baseline tests:

- FBC, renal and liver profile
- Bone profile
- Blood glucose

Regular tests:

- FBC, renal and liver profile as clinically indicated
- Blood glucose and bone profile as clinically indicated

Disease monitoring:

Disease monitoring should be in line with the patient's treatment plan and any other test/s as directed by the supervising Consultant.

DOSE MODIFICATIONS:

- No recommended dose modifications.

Table 1: Dose modification of degarelix in renal and hepatic impairment

Renal Impairment		Hepatic Impairment	
Mild	No dose modification necessary	Mild	No dose modification necessary
Moderate		Moderate	
Severe	No information. Caution advised	Severe	Has not been studied. Caution advised

SUPPORTIVE CARE:

EMETOGENIC POTENTIAL:

- As outlined in NCCP Classification Document for Systemic AntiCancer Therapy (SACT) Induced Nausea and Vomiting Available on the NCCP website

Degarelix: Minimal (**Refer to local policy)**

For information:
Within NCIS regimens, antiemetics have been standardised by the Medical Oncologists and Haemato-oncologists and information is available in the following documents:

- NCCP Supportive Care Antiemetic Medicines for **Inclusion in NCIS** (Medical Oncology) - Available on the NCCP website
- NCCP Supportive Care Antiemetic Medicines for **Inclusion in NCIS** (Haemato-oncology) - Available on the NCCP website

PREMEDICATIONS: None

OTHER SUPPORTIVE CARE:

Calcium and vitamin D supplementation (Refer to local policy)

ADVERSE EFFECTS

- Please refer to the relevant Summary of Product Characteristics (SmPC) for details.

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DRUG INTERACTIONS:

- Current SmPC and drug interaction databases should be consulted for information.

REFERENCES:

1. Klotz L, Boccon-Gibod L, Shore ND, et al. The efficacy and safety of degarelix: a 12-month, comparative, randomized, open-label, parallel-group phase III study in patients with prostate cancer. BJU Int 2008;102(11):1531-1538.
2. Degarelix (FIRMAGON ®) Summary of Product Characteristics. Last updated: 10/01/2020 Accessed May 2025. Available at: https://www.ema.europa.eu/en/documents/product-information/firmagon-epar-product-information_en.pdf

Version	Date	Amendment	Approved By
1	30/05/2018		Prof Maccon Keane
2	27/05/2020	Reviewed.	Prof Maccon Keane
3	30//07/2025	Reviewed. Regimen updated in line with NCCP Standardisation.	Prof Maccon Keane

Comments and feedback welcome at oncologydrugs@cancercontrol.ie.

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